

**Effect Of Seminar Course Based On Dialectical Behavior Therapy Skills &
Social-Emotional Learning On Students Distress Tolerance**

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Abstract

Low emotional tolerance paired with academic stress is deemed as the most general mental condition that students encounter throughout their educational process and is one of the central issues in education. The many stresses put on students through education impact them both inside and outside of the building, in turn being harmful to the same tasks that the students need to do, creating a lose-lose situation for the student. Previous studies showed that Dialectical Behavior Therapy (DBT) can help reduce burnout and decrease the level of stress in adolescents. Additionally, it was shown that Social and Emotional Learning (SEL) can be taught in schools, increasing students' ability to regulate emotions and behavior which leads to positive outcomes. The goal of this study was to survey the effect of a preparatory course based on DBT and SEL on the emotional tolerance of high school freshmen. It was expected that this course would lead to a decrease in stress levels, improvement in emotional tolerance, and students reporting that they utilize more school resources and mindfulness strategies. T-tests revealed that there was no significant improvement in the overall score of the distress tolerance scale after taking the DBT/SEL based course. However, responses to certain questions in the survey reveal an improvement in some aspects of the students distress tolerance. In the future, courses that focus on students' mental health or use DBT or SEL should be integrated into more schools and curriculums to help adolescents grow and strengthen their mental health.

Acknowledgements

The student researcher worked with two mentors. The mentors had taught the Dialectical Behavior Therapy (DBT) skills and the curriculum used in the study more in-depth. Both the mentor and the student's science research teacher had distributed and collected the physical consent forms to maintain anonymity. All paperwork such as research plan/papers, forms, and surveys were checked and feedback was received from the mentors and the science research teachers.

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Introduction

Overview

Today, 61% of high school students report having low emotional tolerance paired with academic stress. This has been deemed to be the most pervasive general mental health condition that students encounter and has been viewed as one of the most central issues in general education (Bennion et al., 2018). Previous studies show that Dialectical Behavior Therapy (DBT) can help reduce burnout and decrease the level of stress in adolescents. Additionally, it is shown that Social and Emotional Learning (SEL) can be taught and nurtured in schools so that students increase their ability to regulate emotions and behavior which leads to positive outcomes (Bennion et al., 2018).

Statement Of Purpose. This study aims to test a course based on both DBT skills and SEL and its ability to impact adolescents all over the world and improve their overall mental health.

Review of Literature

Academic Stress

Academic stress is deemed as the most general mental condition that students encounter throughout their educational process, and it has been viewed as one of the most central issues in general education (Xu J. et al., 2022). Unfortunately, the American high school education system currently maximizes student stress levels among adolescents in an attempt to overall prepare them for the future (Janet et al., 2019). These stresses put on students for high grades, peer validation, domestic problems, and more are all things that impact them outside of the building as well. Ironically, this excessive stress can in turn be harmful to the same tasks that the students need to do creating a lose-lose situation for the student. This excessive stress is also known as burnout (Doak J. et al., 2018).

Mindfulness and DBT Skills

A 2020 study outlined the problem of the high level of aggression and self-harm found among adolescents (Fayaz, et al., 2020). This study examined the effects of meditation and DBT skills on aggression and self-harm among adolescent girls of Kashmir (Fayaz, et al., 2020). The method used to do this was to test 100 girls, aged 14-16, and test them online on their aggression/self-harm levels. The 50 girls who scored the highest were split into a control and experimental group, each with 25 participants. One group was given DBT treatment while the other was given simple breathing treatment. What was found was that learning DBT skills decreased aggression and increased their quality of life by reducing their self-harm (Figure 1) (Fayaz, et al., 2020).

Figure 1: Pre and post test comparisons of adolescent self-harm levels before and after DBT (Fayaz, et al, 2020)

Table 5
Showing Mean, S.D and t value of pre and post test of Anger of Experimental Group

Test (Experimental Group)	N	Mean	Std. Deviation	T	P value
Pretest	36	22.9444	6.45841	2.344*	Significant at 0.05 level of significance
Posttest	36	19.7778	4.89963		

** Significant at $p < 0.01$ * Significant at $p < 0.05$

Table 9
Showing Mean, S.D and t value of pre and post test of Self Harm Experimental Group

Test (Experimental Group)	N	Mean	Std. Deviation	T	P value
Pretest	36	16.055	5.031	3.177**	Significant at 0.01 level of significance
Posttest	36	13.083	2.488		

** Significant at $p < 0.01$

* Significant at $p < 0.05$

DBT for Adolescents (DBT-A)

Another study conducted in 2011 addressed that there were no empirically validated treatments of good quality for adolescents showing suicidality and non-suicidal self-injurious behavior (Fleischhaker, et al., 2011). The goal of this study was to test if Dialectical Behavioral Therapy for Adolescents (DBT-A) was an effective treatment for these patients (Fleischhaker, et al., 2011). The method used to test this was to conduct a 16-24 week outpatient treatment which was then measured by a pre-/post- comparison and a one-year follow-up with the aid of standardized instruments. What was found was that that the interventions were well accepted by the patients and there families, and were associated with improvement in multiple domains including suicidality, non-suicidal self-injurious behavior, emotion dysregulation and depression from the beginning of therapy to the one-year follow-up (Fleischhaker, et al., 2011) (Figure 2).

Figure 2: Review Of Diagnostic Criteria Of BPD One Year After DBT-A (Fleischhaker, et al, 2011)

Table 2 Review of diagnostic criteria of borderline personality disorder before (t1) and one year after therapy (t3) in the pilot study of Dialectical Behavioral Therapy for Adolescents (DBT-A)

Diagnostic DSM-IV Criteria of borderline personality disorder	Number of adolescents satisfying this criterion at the start of therapy (t ₁)	Number of adolescents satisfying this criterion one year after therapy (t ₃)
Frantic efforts to avoid real or imagined abandonment	9	1
A pattern of unstable and intense interpersonal relationships characterized by alternating between extremes of idealization and devaluation	9	2
Identity disturbance: markedly and persistently unstable self-image or sense of self	8	2
Impulsivity in at least two areas that are potentially self-damaging (e. g. spending, sex, substance abuse, reckless driving and binge eating)	8	5
Recurrent suicidal behavior, gestures, threats or self-mutilating behavior	12	10
Affective instability due to a marked reactivity of mood (e. g. intense episodic dysphoria, irritability, or anxiety usually lasting a few hours and only rarely for more than a few days)	12	8
Chronic feelings of emptiness	8	4
Inappropriate, intense anger or difficulty in controlling anger (e. g. frequent displays of temper, constant anger and recurrent physical fights)	4	0
Transient, stress-related paranoid ideation or severe dissociative symptoms)	0	2

Social and Emotional Learning in Education

A study investigated the problem that many children do not receive the needed social and emotional learning in the home or community while schools can serve as an additional support and effective venue through which resilience can be fostered (Duncan, G. J., et al., 2005). The goal of this study was to examine whether students who participated in a curriculum called Strong Start showed a decrease in externalizing and internalizing behaviors and an increase in peer-related prosocial behaviors (Duncan, G. J., et al., 2005). The method used to test this was to enroll one group of students in the Strong Start program and a similar group of students in a regular class and to observe the differences in mental health of students in each class. The findings of this study support the use of Strong Start as a tool for fostering social and emotional competence in students (Duncan, G. J., et al., 2005).

Dialectical Behavior Therapy (DBT) and Social & Emotional Learning (SEL)

Fortunately, Dialectical Behavior Therapy (DBT) is shown to help reduce burnout and decrease the level of stress in adolescents (Figure 3). Another program that uses SEL is a curriculum called StrongStart. A study by Caldarella et al. (2009) was done using this curriculum and showed that SEL was an effective means to reduce emotional and behavioral problems in a school setting for second graders.

Figure 3: The 4 treatment modes of Dialectical Behavior Therapy Skills (Balogun S. K. et al, 2014)



The DBT And SEL Based Course

The course examined in this study was designed to include the learning and understanding of DBT skills (Figure 3). Additionally, this course taught Social and Emotional Learning (SEL), mindfulness, and skills training for emotional problem solving for adolescents to overall improve the mental health of students in high school (Zavala-Berverna et al., 2018). This DBT and SEL-based high school preparatory course is a mandatory class for all incoming freshmen in high school designed to improve the general emotional tolerance and mental health of all students, which could positively impact the students.

Problem Statements

1. Stresses put on students for high grades, peer validation, domestic problems, and more are all things that impact them.
2. A current lack of courses or programs based on DBT/SEL skills that address this that could be added to schools.

Objectives

1. To test the effect of a high school freshman preparatory course based on DBT skills and SEL on students enrolled.
2. Find how these courses impact students' emotional tolerance, distress levels, and ability to utilize a variety of coping skills.

Hypotheses

1. After taking a course based on DBT and SEL learning, distress levels will lessen due to the improvement of their emotional tolerance.

2. More people will use more mindfulness and coping strategies in everyday life after learning DBT skills through Social and Emotional Learning (SEL) leading to an improvement in emotional tolerance and decreased distress levels.

Methodology

Role of Student vs. Mentor

The role of the mentor was to assist in the editing and revising of the survey being handed out to the students. The mentor also aided in the distribution of the survey to the students taking the course. Additionally, they gave advice and guidance to the students throughout the project. It took 6-8 weeks to create and distribute the survey after institutional review board (IRB) approval was obtained. The student was responsible for distributing the survey at the end of the 2022 school year to freshmen students who have taken the DBT and SEL-based courses. Then, the student spent 2-3 months distributing the survey once again by reaching out to numerous local DBT groups and support groups for freshmen. The student was also responsible for data analysis after responses were submitted.

Survey Creation

The survey was created using Google Forms, which consisted of multiple sections, including those sections listed below:

Introduction to Survey. This section consisted of an introductory message, shown in Figure 4, that illustrated important information provided to the participants. This information included important details regarding the survey such as the length of the survey, confidentiality, and helpful information for when the participants were completing the survey (Figure 4).

Figure 4: Important information for participants is provided (Figure by Akshyasree Ravi).

Coping Skills Learned from a DBT Based Course in High School

Thank you for participating in my study about the effect of coursework in Dialectical Behavior Therapy (DBT) on incoming high school student's coping ability. My name is Akshyasree Ravi and I am a student in the Science Research Program at Yorktown High School.

The goal of this work is to observe how a DBT skills-based high school course effects students' knowledge of various coping skills.

Listed below is important information to know before beginning the survey:

- This survey will take approximately 5-10 minutes to complete.
- This survey is for all high school students.
- Participant age range is 13-17 and parental consent will be collected via this Google Form.
- If a parent wishes to see this survey before the participant takes it, please email akshyasree.ravi@yorktown.org and/or mrubeo@yorktown.org. We will send you the PDF version of the full survey and be happy to answer any questions you may have.
- Your data will be kept completely ANONYMOUS to the researcher.
- You are allowed to stop your participation in the survey at ANY time.
- If you would like to know the results of this study once it is completed, please send a separate email to akshyasree.ravi@yorktown.org with "Mental Health Course Survey" as the subject, I will be happy to share my results.
- This survey will be beneficial to society because it will provide insight into the influence of mental health courses for teenagers on their ability to thrive in high school and beyond. There is a minimal risk of a data breach since this is a digital survey. However, to protect confidentiality, no IP addresses, emails, or specifically identifiable information will be collected from participants.

If you have any more questions regarding the work, you can reach out to me at akshyasree.ravi@yorktown.org or my Science Research teacher Mr. Rubeo at mrubeo@yorktown.org.

Age and Consent Section. In keeping with the IRB consent procedures, this section asked the participants to choose their ages. All participants were minors in the range of 13-16 years old. The purpose of the consent questions was to make sure a parent approved since the participants were not above 18 years old.

Demographics Section. This section was included to allow participants to indicate whether they have taken the DBT & SEL-based course. In addition, they were asked about their age, gender, and ethnicity. The goal of this section was to help in analyzing results and to see trends between the professional DBT groups and the DBT & SEL-based course. All of this information was used by the student in the data analysis portion of the study.

Husker Discovery Section. The students were asked to indicate how often they have used a variety of DBT skills in the past month using Likert-type scales (Figure 5).

Figure 5: Example of DBT Skills questions section (Figure by Akshyasree Ravi).

For each question choose from the following alternatives: 1- Strongly Disagree, 2 - Mildly Disagree, 3 - Feel Neutral, 4 - Mildly Agree, 5 - Strongly Agree

	1	2	3	4	5
I have tried using Mindfulness Skills (Wise Mind, "What", "How").	☐	☐	☐	☐	☐
I have tried using Emotional Regulation Skills (Check the Facts, Opposite Action, Problem Solving, ABC PLEASE, The Wave Skill).	☐	☐	☐	☐	☐
I have tried using Distress Tolerance Skills (Crisis Survival Skills, ACCEPTS, Self-Sooth, IMPROVE, TIP, Pros and Cons).	☐	☐	☐	☐	☐

Distress Tolerance Scale Survey Section. The DTSS is a validated and previously published 15-item self-report measure of emotional distress tolerance (Simons, J. S., & Gaher, R. M., 2005). It was taken by both the students who are taking or have taken the DBT and SEL-based course. Using Likert-type scales, the students were asked to indicate how often they felt or thought a certain way in the past month (Figure 6).

Figure 6: Example of emotional tolerance questions from the Distress Tolerance Scale Survey section. (Figure by Akshyasree Ravi).

Coping Ability Questions					
For each question choose from the following alternatives: 1- Strongly Disagree, 2 - Mildly Disagree, 3 - Feel Neutral, 4 - Mildly Agree, 5 - Strongly Agree					
	1	2	3	4	5
I feel confident in my studying abilities.	☐	☐	☐	☐	☐
My schoolwork and supplies are very organized.	☐	☐	☐	☐	☐
I rarely get nervous or anxious before an exam.	☐	☐	☐	☐	☐
I am comfortable around my peers.	☐	☐	☐	☐	☐

Survey Distribution

DBT And SEL Based Course. The student's mentor was asked to distribute the survey to their students before and after they had taken the course.

Data Analysis

Statistical analysis was conducted using DataClassroom, and the student conducted several t-tests, an ANOVA, and a linear regression analysis to determine the strength of correlation between several factors including age, gender, and distress tolerance. The student researcher analyzed the statistical results in order to find which trends were statistically

significant, with a p value of less than 0.005. Trends were further analyzed to determine how effective the course was in building better emotional tolerance.

Results

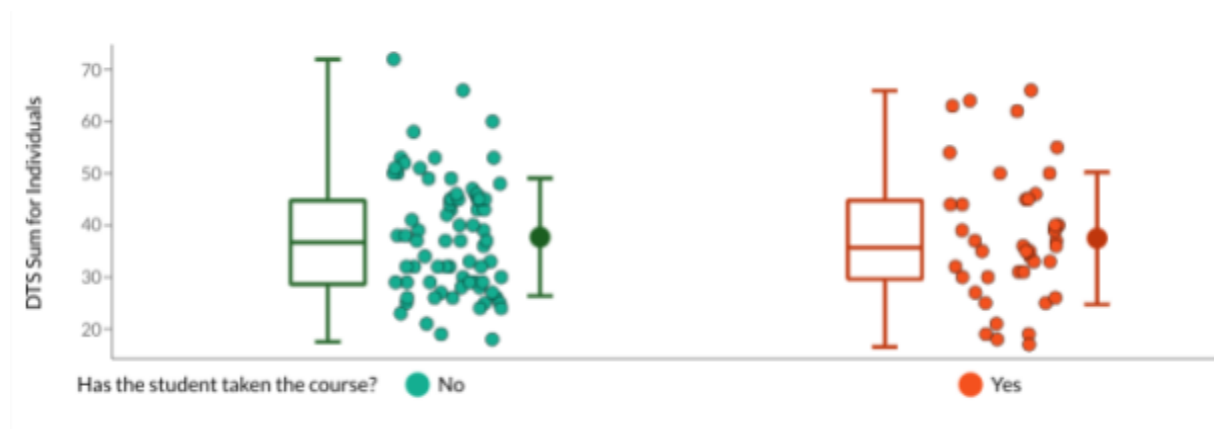
Demographics

The survey has received 118 responses. The age of the participants ranged from 13 to 16-year-old children with most of them being freshmen in high school. A total of 48.2% of the responders were male and 47.5% were female. Additionally, one respondent was trans/non-binary, one was genderfluid, and two preferred not to answer. A total of 64% of participants were white/caucasian which accounted for 76 of the respondents. The rest consisted of 27.1% Asian respondents, 11% Hispanic/Latino/Spanish, and 2.5% Black/African American. The ages of the participants ranged from 13-16 with the largest group at age 14 (48.3%) and the smallest at age 13 (8.5%).

Effect of DBT Skills/SEL Course On Student's Emotional Tolerance

The Distress Tolerance Stress Scale was used to measure students' Emotional Tolerance before and after taking the DBT skills and SEL-based course. Overall, 67 (56.7%) respondents had not completed the course and 51 (43.2%) respondents had. A t-test was used to compare the sums of both groups' response to the Distress Tolerance Scale (DTS) to measure whether distress tolerance of students had changed. The p-value was derived from the t-score and the DF to be 0.93. This can be interpreted as meaning no evidence that the groups might be different. A box plot of these statistics can be seen in Figure 7 below.

Figure 7: Box plot of T-test results show little difference in DTS sums before and after students have taken the DBT/SEL based course. (Made with DataClassroom)



Although the overall results of the DTS scores reveal this outcome, sums of specific questions asked in the survey shows a strong difference between the students that had taken the course and students that have not. For example, question 5 of the distress tolerance survey asks students to rate their level of agreeance for the question “There's nothing worse than feeling distressed or upset” on a likert scale of 1-5. A rating of 1 is to strongly disagree and a rating of 5 is to strongly agree. When the question 5 DTS scores of students were compared between students who had and had not taken the DBT/SEL based course, the p-value was derived from the t-score and the DF to be 0.04. This can be interpreted as meaning that there is strong confidence that the groups are different. A box plot of these statistics can be seen in Figure 8 below.

COVID-19 quarantine and did not get the same experience as people who took the course in 2022. The difference in learning could impact how the students respond and learn from the course as well as their stress levels. Also, a potential limitation was the grade levels in the participants who responded to having completed the course. The participants who had completed the course ranged from grade levels 10 to 12 which could lead to inaccurate collection of stress levels.

Application

The potential impact of the findings from this survey can positively impact the greatest amount of lives by living and dealing with stress in education. The survey results can also be used to show the stress levels change between different ages, races, and genders. This information can be used to make more mental-health based courses in schools catered to more students in schools. This research can be improved by finding other aspects of therapy that can be implemented into education.

Future Research

As evident by the results in this study, participating in a DBT and SEL based course is shown to be somewhat effective in the improvement of students distress tolerance skills. Future research could involve looking further into the emotional impact of the DBT/SEL based course and other types of therapy skills that could be integrated into education. Additionally, contacting local support groups could aid in comparing DBT groups and DBT skills in schools to find similarities and differences in how kids are affected. For example, if a student's emotional tolerance is improved more by a certain aspect of DBT in a local support group, it can be implemented in education through the curriculum or a course. Other future research includes expanding on the survey by inserting questions about specific DBT skills usage to be answered

by both students before and after taking the course to study if individuals cope effectively or less effectively. Also by expanding, greater correlations can be found between different factors going into different skills and what seemed to affect students the most. Another possible avenue for future research could include collecting data from the same students directly before and after completing the same DBT/SEL based course during the same timeframe.

Conclusion

The objective of this study was to test the effect of a high school freshman preparatory course based on DBT skills and SEL on students enrolled. This was met by using the Distress Tolerance Scale (DTS) in a Google Form survey. A total of 56.8% of participants responded before completing the course and 43.2% of respondents had completed the course. The survey was developed using Google Forms and distributed via email, Google Classroom, and Discord. The results from the study suggest that there is no significant change in students' distress tolerance before and after taking the DBT/SEL based course. However, a couple specific questions asked in the Distress Tolerance Scale showed significant changes in distress tolerance of the students after taking the course. The first hypothesis that after taking a course based on DBT and SEL learning, students' distress levels will lessen due to the improvement of their emotional tolerance was overall refuted but supported in some instances. These results can be used in a substantial way by many schools to create a course that is more likely to improve the emotional tolerance and overall mental health of their students. For example, using the specific DBT skills that worked better for students or testing different skills taught in therapy on more students could help build a course that helps improve the mental health of adolescents around the globe. Poor mental health impacts millions of children across the country and more research needs to be done to solve this epidemic.

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